NexaWorks Medical - Proposal

Response to RFP MCH-EMR-2026-01 | Submitted 12 September 2026 | Contact: tenders@nexaworks.example

1. Executive summary

NexaWorks Medical proposes NexaCare, a configurable EMR platform used by multi-hospital groups, delivered through a phased and clinician-led programme. We understand that for MCH the greatest risks are not software features but clinical adoption, interface readiness and stability during go-live. Our proposal is built around controlling those risks.

2. Proposed solution and approach

NexaCare combines a configurable clinical core with a commercial integration engine supporting HL7 v2, FHIR R4 and DICOM through a PACS gateway. ABDM milestones M1 to M3 are already certified. The platform has been tested with 5,000 concurrent users and is deployed in a primary data centre with a disaster-recovery site (RPO 15 minutes, RTO 2 hours, 99.9% availability). Order sets and clinical pathways are configured with MCH clinicians rather than imposed as templates.

3. Implementation plan, timeline and team

The 9-month plan uses a pilot ward, a pilot hospital and two further waves, each gated by acceptance criteria agreed in advance with MCH. The 24-person core team includes named leads: Programme Director (Rohan Mehta), Clinical Informatics Lead (Dr. Kavita Rao, a practising physician), Integration Lead, Data Migration Lead and Training Lead. MCH nominates a clinical champion per department.

Phase	Period	Milestone / acceptance gate
Mobilise	Week 1-4	Governance in place, workshops scheduled, interface catalogue draft
Design	Week 5-10	Signed-off clinical workflows per department (14 departments)
Build and integrate	Week 8-20	All 12 interfaces passing end-to-end tests with sample data
Migration rehearsals	Week 14-26	Two rehearsals with 99.5% reconciliation of patient records
Pilot ward and Hospital 1	Week 22-26	Go/no-go review; safety sign-off by CMIO
Hospitals 2, 3, 4	Week 28-36	One go-live every 3 weeks; each gated on previous hypercare exit
Hypercare	8 weeks per site	Daily huddles, floor-walkers, defect SLA tracking

Key risk	Likelihood / impact	Mitigation and owner
Legacy LIS interface delay	Medium / High	Early interface sandbox in week 3; fallback file-based exchange (Integration Lead)
Low clinician availability	High / Medium	Protected design-workshop slots in contract; backfill for super-users (Clinical Lead)
Migration data-quality issues	Medium / High	Two rehearsals, clinical validation of samples (Data Migration Lead)
Adoption resistance	Medium / High	1 super-user per 25 users; simulation lab; floor-walkers (Training Lead)

4. Price

Cost component	INR lakh	Basis
Platform subscription (5 years)	510	Per-bed licence, 1,200 beds, includes patient portal
Implementation and configuration	270	Fixed price, 9-month programme
Data migration (5 years history)	120	Includes two full rehearsals and clinical validation
Interfaces (LIS, PACS, pharmacy, ABDM)	150	Up to 12 interfaces; additional at INR 4 lakh each

Cost component	INR lakh	Basis
Training and super-user programme	70	3,500 users, simulation lab, floor-walkers
Support and resident engineers (5 years)	370	24x7 support; resident engineers in year 1
Total 5-year cost	**1,490**	**= INR 14.90 crore**

Pricing assumptions

- Prices are in INR lakh, exclusive of GST; valid for 90 days from submission.
- Hosting in two Indian data centres (primary and disaster recovery) is included.
- Annual support escalation capped at 3% from year 2; price protection for additional beds at the same per-bed rate.
- Excluded: client-side hardware and hospital network upgrades; PACS third-party licences.

5. Security, compliance and risk controls

- Certification: ISO/IEC 27001:2022 certified; SOC 2 audit planned for 2027.
- Encryption in transit (TLS 1.2+) and at rest (AES-256); role-based access with multi-factor authentication.
- Audit trail of all record access, retained for 7 years; data hosted in India.
- Mapping of platform controls to DPDP Act, 2023 provided in Annex A of the contract; annual penetration test by an external auditor.

6. Support model, experience and references

Priority	Response	Resolution target	Coverage
P1 - system down / patient safety	15 minutes	2 hours	24x7, on-site within 4 hours
P2 - major function degraded	30 minutes	8 hours	24x7
P3 - minor issue	4 hours	3 working days	Business hours

Support is tiered (L1 service desk, L2 application, L3 engineering). A named Customer Success Manager runs monthly service reviews, and resident engineers are placed at each hospital for 12 months after go-live. Service credits of up to 15% of the quarterly support fee apply if SLAs are missed. A knowledge base and in-app guided help reduce ticket volume.

NexaWorks has completed 8 hospital deployments over 6 years. References: Riverside Medical Centre (480 beds, live 2024), Unity Hospitals Group (3 hospitals, 750 beds, live 2025), Sunrise Women's Hospital (200 beds, live 2023).